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AWR 201

Process Work 3: Research Proposal Draft

1. Treatment of Alcoholism

2. Alcohol use disorder (AUD) affects roughly 29 million Americans, yet only about 7% will ever receive treatment in a given year. Even for those who do get treatment, relapse rates remain stubbornly high, often cited around 40-60% within the first year. This problem hit close to home for me, as I have had personal experience with alcoholism. It begs the question of what are ways to actually treat AUD? The two main approaches are medication-assisted treatment (MAT), using FDA-approved drugs like naltrexone and acamprosate, and behavioral therapy, like cognitive-behavioral therapy (CBT). Both are evidence-based, but the question of whether combining them produces better outcomes than either alone is not as settled as you would think.

One of the most important studies to address this question is the COMBINE Study by Anton et al., published in JAMA in 2006. This study is the largest pharmacotherapy trial conducted for AUD in the United States and randomized 1,383 alcohol-dependent patients to nine different treatment combinations involving naltrexone, acamprosate, CBT-based behavioral intervention (CBI), and medical management. Interestingly, the researchers found that "patients receiving naltrexone plus medical management, CBI plus medical management and placebos, or both naltrexone and CBI plus medical management had higher percent days abstinent (80.6, 79.2, and 77.1, respectively) than the 75.1 in those receiving placebos and medical management only" (Anton et al.). What is striking here is that combining naltrexone with CBI did not produce better results than either treatment alone, contradicting the common assumption that more treatment is always better.

Furthermore, these findings matter because clinicians and patients are constantly making decisions about treatment plans, often under the assumption that the combined approach is the gold standard. Moreover, if the COMBINE Study is correct, healthcare systems may be wasting resources on the combination when either treatment alone could achieve similar outcomes. Additionally, with alcohol-related deaths in the U.S. rising to over 178,000 per year (CDC), getting treatment right is not just an academic question, it is a matter of life and death. Understanding which approach works best, and for whom, is essential for clinicians to design more effective treatment protocols going forward.

3.

- Does combining cognitive-behavioral therapy with medication-assisted treatment produce significantly lower relapse rates in patients with alcohol use disorder than either treatment alone?
- How do patient-specific factors (genetic markers, severity of AUD, coexisting psychiatric conditions, social support) predict which AUD treatment approach will be most effective for that individual?
- How does the medical and psychological treatment of alcohol use disorder differ between adolescents and adults, and what explains those differences?
- How do long-term outcomes (5+ years post-treatment) compare between AUD patients treated with MAT only, CBT only, or a combined approach?

- Why are FDA-approved medications for alcohol use disorder (naltrexone, acamprosate, disulfiram) so significantly underprescribed by primary care physicians despite proven efficacy?